

1. Administrative & Study Identification

Full Study Title: A 96-week, two-arm, randomized, single-masked, multi-center, phase III study assessing the efficacy and safety of brolucizumab 6 mg compared to panretinal photocoagulation laser in patients with proliferative diabetic retinopathy **Short Title/Acronym:** CONDOR **Protocol Number:** CRTH258D2301 **NCT Number:** NCT04278417 **Posting ID:** 807690768812278036 **Trial Status:** COMPLETED **Sponsor/Company Name:** Novartis **Investigational Product:** Brolucizumab 6 mg (Trade Name: Beovu, ATC Code: S01LA06) **Phase of Development:** Phase III **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of brolucizumab compared to panretinal photocoagulation laser (PRP) in patients with proliferative diabetic retinopathy (PDR) with respect to the change in best corrected visual acuity (BCVA) at Week 54. This evaluation will provide information that brolucizumab is non-inferior to PRP. **Secondary Objectives:** To evaluate the maintenance of visual acuity through Week 96, the reduction in signs of PDR, the reduction of swelling in the center of the macula, and the overall safety and tolerability profile.

2.2 Background & Rationale Proliferative diabetic retinopathy (PDR) is an advanced retinopathy due to diabetes mellitus characterized by the formation of new abnormal and fragile vessels in the retina. Panretinal photocoagulation (PRP) is the traditional primary treatment for PDR. This study aims to demonstrate that brolucizumab, an anti-vascular endothelial growth factor (anti-VEGF) injection, is non-inferior to PRP in preserving visual acuity while potentially offering greater benefits in preventing complications such as macular edema.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Panretinal photocoagulation laser - PRP) **Blinding:** Single-masked **Randomization:** 1:1 ratio

3.2 Planned Enrollment & Duration Target **SN\$:** 689 participants **Number of Sites:** 121 locations across 15 countries **Estimated Study Start:** Nov 19, 2020 **Estimated Primary Completion:** Oct 30, 2023

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Signed informed consent must be obtained prior to participation. 2. Adult participants with Proliferative Diabetic Retinopathy (PDR) diagnosis with no previous PRP treatment in the study eye. 3. Diagnosis of type 1 or 2 Diabetes Mellitus (DM) and HbA1c $\leq 12\%$ at screening. 4. DM treatment stable for at least 3 months. 5. Able to complete adequate fundus photographs and retinal images. 6. Best Corrected Visual Acuity (BCVA) ETDRS letter score of ≥ 34 ETDRS letters (Snellen equivalent 20/200) at Screening / Baseline in the study eye.

4.2 Exclusion Criteria 1. Concomitant conditions or ocular disorders in the study eye at Screening or Baseline that could compromise a response to study treatment. 2. Presence of diabetic macular edema (DME) in the study eye at Screening or Baseline. 3. Active infection or inflammation in the study eye. 4. Uncontrolled glaucoma (IOP > 25 mmHg). 5. Intravitreal anti-VEGF treatment within 6 months prior to the study. 6. Treatment with intraocular corticosteroids or systemic anti-VEGF therapy at any time. 7. End stage renal disease requiring dialysis or kidney transplant. 8. Uncontrolled blood pressure.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Brolucizumab 6 mg administered as 3 initial loading doses every 6 weeks (q6w), followed by maintenance doses every 12 weeks (q12w) through Week 90. From Week 48 onwards, the treatment interval could be extended by 6 weeks at a time (up to 24 weeks) or reverted to q12w/q6w if the disease worsened. *Control Arm:* PRP initial treatment in 1-4 sessions up to Week 12, followed by additional PRP treatment (may split into 2-4 sessions) as needed up to Week 90. **Route of Administration:** Intravitreal injection. **Duration of Treatment:** Up to 96 weeks. Visits occurred every 6 weeks throughout the study, regardless of treatment or not.

5.2 Concomitant & Prohibited Therapy Permitted: Standard medical management for systemic diabetes and treatments for the untreated (fellow) eye. **Prohibited:** Systemic anti-VEGF therapy, intraocular corticosteroids, and other intravitreal anti-VEGF agents in the study eye.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -14 to 0 Informed Consent, Medical History, Visual Acuity (ETDRS) Testing Baseline Day 0 Randomization, First Brolucizumab Injection or PRP Session

| **Interim** | Every 6 Weeks | Visual Acuity Assessment, Ocular Exam, Adverse Event Monitoring | | **End of Study** | Week 96 | Final BCVA Assessment, Exit Interview, Safety Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Change from baseline in Best Corrected Visual Acuity (BCVA) at Week 54 for the Study Eye.

Measurement Method: Visual function of the study eye was assessed using Early Treatment Diabetic Retinopathy Study (ETDRS) visual acuity testing charts. Minimum and maximum possible scores are 0-100 respectively. A higher score represents better functioning.

7.2 Secondary Endpoints 1. Number and percentage of subjects with no Proliferative Diabetic Retinopathy (PDR) at Week 54 for the Study Eye. 2. Number and percentage of subjects with no Proliferative Diabetic Retinopathy (PDR) at Week 96 for the Study Eye. 3. Number and percentage of subjects with Center-involved Diabetic Macular Edema (CI-DME) up to Week 54 for the Study Eye.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection of ocular (e.g., intraocular inflammation) and non-ocular AEs (e.g., hypertension, upper respiratory tract infections).

Serious Adverse Events (SAEs): Standard reporting timeline, typically 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal/External safety monitoring mechanisms managed by Novartis.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for all AE/SAE evaluations.

Sample Size Justification: Powered to demonstrate non-inferiority of brolocizumab compared to PRP with respect to BCVA change.

Handling of Missing Data: Last Observation Carried Forward (LOCF) was used for the imputation of missing values.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite monitoring and central verification of ETDRS assessments. **Audit Trail:** eCRF locking, tracking of central reading center (CRC) image grading, and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: CRTH258D2301 **Registry Number:** NCT04278417 **Posting ID:** 807690768812278036 **Sponsor/Lead Organization:** Novartis **Study Title:** CONDOR **Official Regulatory Title:** A 96-week, two-arm, randomized, single-masked, multi-center, phase III study assessing the efficacy and safety of brolucizumab 6 mg compared to panretinal photocoagulation laser in patients with proliferative diabetic retinopathy.

12. Clinical Framework (PDR Specific)

Primary Condition: Proliferative Diabetic Retinopathy (PDR) **Diagnosis Threshold:** BCVA ETDRS letter score ≥ 34 **Medical Methodology:** Anti-VEGF Intravitreal Therapy versus Panretinal Photocoagulation Laser **Optimization Target:** Best Corrected Visual Acuity (BCVA) preservation and reduction of PDR signs

13. Study Procedures & Methodology

Management Pathway: Standard clinical ophthalmic pathway for PDR **Education Protocol (HCP):** Protocol training and central reading center imaging guidelines **Education Protocol (Patient):** Onsite training and post-intravitreal injection/laser care instructions **Quality Audit Mechanism:** Central reading center (CRC) evaluation of 7-field color fundus photography and SD-OCT imaging

14. Observation & Follow-Up Schedule

Total Duration: 96 Follow-up weeks **Onsite Visit Cadence:** Every 6 weeks (q6w) throughout the study **Remote Monitoring Frequency:** Standard AE reporting as needed between 6-week intervals **Checklist Review Interval:** Every 6 weeks at regular onsite visits

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	06-Apr-2026				
Clinical Operations Lead	[Name]	_____	06-Apr-2026		Lead		
Statistician	[Name]	_____	06-Apr-2026				